# RITES TOOL

***Explanatory Design***: determines whether an intervention produces the expected result under ideal research circumstances and are intended to provide evidence on the *efficacy* of an intervention.

***Pragmatic Design***: determines the degree of beneficial effect under "real world" clinical conditions and are intended to provide evidence on the *effectiveness* of an intervention.

| **Criteria** | | **Descriptor for trial with maximum *effectiveness* (pragmatic)** | **Descriptor for trial with maximum *efficacy* (explanatory)** |
|---|---|---|---|
| **Participants** | **Characteristics/Description*** | The participants are representative of the population who would receive the experimental intervention if it was part of usual care. ** They are of similar age, severity of illness and comorbidities to those patients who would be candidates for the intervention in usual care setting. They reflect diversity along parameters that could impact adherence. | The participants are a homogenous population and are markedly different from those seen in usual care. May have been deliberately selected to comply with treatment, respond to treatment or demonstrate the efficacy of the experimental intervention. There may be other exclusions that would not be seen in usual care ** (e.g. exclusion of participants with comorbidities). |
| | **Eligibility** | How similar are the participants to those who would receive the intervention if it was part of usual care? | There are many exclusions (e.g. those who don't comply, respond to treatment, or are not at high risk for the primary outcome, etc.), or the trial uses many selection tests not used in usual care. |
| | **Recruitment** | Recruitment is through usual appointments or clinics | Recruitment is through targeted information letters, advertising in newspapers, radio plus incentives and other routes that would not be used in usual care. |
| **Trial Characteristics** | **Setting** | The overall setting of the trial is similar to usual care – and includes diverse sub-settings common in usual care for this intervention (e.g. primary care, specialized care) | The setting is selected to maximize the ability to carry out the trial and identify an intervention effect if there is one. The setting may be more specialized than the setting in which the intervention would be delivered in usual care (e.g. a single center, specialized clinics or specialized academic centres or only providers with high levels of experience). |

| | | | |
|---|---|---|---|
| | **Organization** | The resources, provider expertise and the organization of care delivery in the intervention arm are identical to usual care | The trial increases staff levels, gives additional training, requires more than the usual experience or certification, or certification and increase resources. |
| | **Comparison intervention** | The control typically is applied by the full range of practitioners and in the full range of clinical settings, with only ordinary attention to the training, experience and performance of the practitioners. | The practitioner expertise is standardized in the control to maximize the chances of detecting whatever comparative benefits the experimental intervention might have |
| **Flexibility** | **Delivery** | Flexibility in the intervention is identical to that in usual care | Intervention delivery is less flexible than usual care. There is a strict protocol, monitoring and measures to improve compliance, with specific advice on allowed co-interventions and complications. |
| | **Adherence** | There is no more than usual encouragement to adhere to the intervention | Exclusion based on adherence and measures to improve adherence if found wanting. |
| | **Practitioner adherence to study protocol** | There is unobtrusive (or no) measure of practitioner adherence and no special strategies to maintain or improve it | There is close monitoring of how well the participating clinicians and centres are adhering to even the minute details in the trial protocol and "manual of procedures". |
| | **Flexibility of the control** | "Usual practice" or the best available alternative management strategy, offering practitioners considerable leeway in deciding how to apply it | Restrictive flexibility of the comparison intervention |
| **Clinical relevance of experimental and comparison interventions** | **Clinical Relevance** | Both the experimental and comparison intervention have the potential to be "best practice" | One or both of the experimental and comparison intervention is not a clinically relevant treatment (e.g. placebo, sub-clinical doses) |
| | **Primary outcome** | The primary outcome is of obvious importance to participants. | The primary outcome uses a surrogate, physiological outcome, central adjudication or uses assessment expertise that is not available in usual care, or the outcome is measured at an earlier time than in usual care. |
| | **Primary analysis** | The most pragmatic approach is to use intention to treat with all available data | The most explanatory analysis is one that excludes ineligible post-randomization participants and includes only those following the treatment protocol. |

* Participants included in the study should be judged according to whether they are representative of the general population of those with the condition of interest who would receive usual care in the geographic area in which the trial is carried out

** Usual care describes the type of care routinely received by patients in the geographic area in which the trial is carried out. The care provided can vary by patient characteristics (eg. age, sex, education), the setting (inpatient/outpatient, primary care/specialized care) in which the patient is seen, individual providers and insurance plans.

## 5-point Likert Scale:

**1 =** Strong Emphasis on Efficacy
**2 =** Rather strong emphasis on efficacy
**3 =** Balanced emphasis on both efficacy and effectiveness
**4 =** Rather strong emphasis on effectiveness
**5 =** Strong emphasis on effectiveness

**Study Title:** A Parent Treatment Program for Preschoolers with Obesity: A Randomized Control Trial

**Authors:** Ek A., Chamberlain, K., Sorjonen, K., Hammar, U., Malek, M., Sandvik, P., Somaraki, M., Nyman, J., Lindberg L., Nordin, K., Ejderhamn, J., Fisher, P., Chamberlain, P., Marcus, C., Nowicka, P.

**Reviewer:** SN, CD

| | Strong Emphasis on Efficacy ❶ | Rather strong emphasis on efficacy ❷ | Balanced emphasis on both efficacy and effectiveness ❸ | Rather strong emphasis on effectiveness ❹ | Strong emphasis on effectiveness ❺ |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Trial Setting** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☒ | ☐ |

**Study Title:** Efficacy of a Community- Versus Primary Care-Centered Program for Childhood Obesity: TX CORD RCT

**Authors:** Butte, N., Hoelscher, D., Barlow, S., Pont, S., Durand, C., Vandewater, E., Liu, Y., Adolph, A., Pérez, A., Wilson, T., Gonzalez, A., Puyau, M., Sharma, S., Byrd-Williams, C., Oluyomi, A., Huang, T., Finkelstein, E., Sacher, P., Kelder, S.,

**Reviewer:** SN, CD

| | Strong Emphasis on Efficacy ❶ | Rather strong emphasis on efficacy ❷ | Balanced emphasis on both efficacy and effectiveness ❸ | Rather strong emphasis on effectiveness ❹ | Strong emphasis on effectiveness ❺ |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Trial Setting** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☐ | ☒ |

**Study Title:** Two-year follow-up of a primary care-based intervention to prevent and manage childhood obesity: the High Five for Kids study

**Authors:** Sheryl L. Rifas-Shiman, Elsie M. Taveras, Steven L. Gortmaker, Katherine H. Hohman, Christine M. Horan, Ken P. Kleinman, Kathleen Mitchell, Sarah Price, Lisa A. Prosser and Matthew W. Gillman

**Reviewer:** SN, CD

| | Strong Emphasis on Efficacy 1 | Rather strong emphasis on efficacy 2 | Balanced emphasis on both efficacy and effectiveness 3 | Rather strong emphasis on effectiveness 4 | Strong emphasis on effectiveness 5 |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Trial Setting** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☒ | ☐ |

**Study Title:** Clinic and Home-Based Behavioral Intervention for Obesity in Preschoolers: A Randomized Control Trial

**Authors:** Stark, L., Filigno, S., Bolling, C., Ratcliff, M., Kichler, J., Robson, S., Simon, S., McCullough, M., Clifford, L., Stough, C., Zion, C., Ittenbach, R.

**Reviewer:** SN, CD

| | Strong Emphasis on Efficacy ❶ | Rather strong emphasis on efficacy ❷ | Balanced emphasis on both efficacy and effectiveness ❸ | Rather strong emphasis on effectiveness ❹ | Strong emphasis on effectiveness ❺ |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Trial Setting** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☒ | ☐ |

**Study Title:** A pilot randomized controlled trial of a behavioral family-based intervention with and without home visits to decrease obesity in preschoolers (2014)

**Authors:** Stark, L., Clifford, L., Towner, E., Filigno, S., Zion, C., Bolling, C., & Rausch, J.

**Reviewer:** SN, RG

| | Strong Emphasis on Efficacy ❶ | Rather strong emphasis on efficacy ❷ | Balanced emphasis on both efficacy and effectiveness ❸ | Rather strong emphasis on effectiveness ❹ | Strong emphasis on effectiveness ❺ |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Trial Setting** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☐ | ☒ |

**Study Title:** A pilot randomized controlled trial of a clinic and home-based behavioural intervention to decrease obesity in preschoolers (Stark 2011)

**Authors:** Stark, L., Spear, S., Boles, R., Kuhl, E., Ratcliff, M., Scharf, C., Bolling, C. & Rausch, J.

**Reviewer:** SN, RG

| | Strong Emphasis on Efficacy ❶ | Rather strong emphasis on efficacy ❷ | Balanced emphasis on both efficacy and effectiveness ❸ | Rather strong emphasis on effectiveness ❹ | Strong emphasis on effectiveness ❺ |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Trial Setting** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☒ | ☐ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☐ | ☒ |

**Study Title:** Efficacy of family-based weight control program for preschool children in primary care (Quattrin 2012)

**Author:** Quattrin, T., Roemmich, J., Paluch, R., Yu, J., Epstein, L., & Ecker, M.

**Reviewer:** SN, RG

| | Strong Emphasis on Efficacy 1 | Rather strong emphasis on efficacy 2 | Balanced emphasis on both efficacy and effectiveness 3 | Rather strong emphasis on effectiveness 4 | Strong emphasis on effectiveness 5 |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Trial Setting** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Flexibility of Intervention** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☐ | ☒ |

**Study Title:** Can a dairy-rich diet be effective in long-term weight control of young children

**Author:** Kelishadi, R., Zemel, M., Hashemipour, M., Hosseini, M., Mohammadifard, N. & Poursafa, P.

**Reviewer:** SN, RG

| | Strong Emphasis on Efficacy 1 | Rather strong emphasis on efficacy 2 | Balanced emphasis on both efficacy and effectiveness 3 | Rather strong emphasis on effectiveness 4 | Strong emphasis on effectiveness 5 |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Trial Setting** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Flexibility of Intervention** | ☐ | ☐ | ☒ | ☐ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☒ | ☐ |

**Study Title:** A child-centered health dialogue for the prevention of obesity in child health services in Sweden – A randomized controlled trial including an economic evaluation

**Authors:** Derwig, M., Tiberg, I., Björk, J., Welander Tärneberg, A., & Kristensson Hallström, I.

**Reviewer:** SN, RG

| | Strong Emphasis on Efficacy 1 | Rather strong emphasis on efficacy 2 | Balanced emphasis on both efficacy and effectiveness 3 | Rather strong emphasis on effectiveness 4 | Strong emphasis on effectiveness 5 |
|---|---|---|---|---|---|
| **Participant Characteristics** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Trial Setting** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Flexibility of Intervention** | ☐ | ☐ | ☐ | ☒ | ☐ |
| **Clinical relevance of experimental and comparison intervention(s)** | ☐ | ☐ | ☐ | ☒ | ☐ |